

CLINICAL FEATURES

Erythematotelangiectatic rosacea (ETR) is characterized by persistent facial erythema and flushing, telangiectases, central face edema, burning and stinging sensations, skin roughness or scaling, or any combination of these features. Mild, moderate, and severe subtypes are recognized.

In contrast, papulopustular rosacea (PPR) presents with persistent central facial erythema accompanied by papules and pustules that predominantly affect convex areas of the face. Mild, moderate, and severe forms are also distinguished in this subtype. Burning and stinging occur less frequently in PPR compared to ETR, and flushing tends to be less severe. In both subtypes, facial erythema typically spares the periorbital regions. Edema may range from mild to severe; when severe, it can manifest as solid facial edema with plaque-like morphology, most commonly on the forehead and glabella, and less frequently on the eyelids and upper cheeks.

Phymatous rosacea is marked by patulous follicular orifices, thickened skin, nodularities, and irregular surface contours, primarily in convex facial areas. Subtypes range from mild to severe. Rhinophyma (nose involvement) is the most common presentation, but phymatous changes can also occur on the chin (gnathophyma), forehead (metophyma), eyelids (blepharophyma), and ears (otophyma). Women rarely develop phyma, possibly due to hormonal factors, but may exhibit sebaceous hyperplasia with thickened skin and enlarged follicular openings.

Ocular rosacea may precede cutaneous symptoms in up to 20% of cases. In approximately half of patients, eye symptoms follow skin involvement. A minority experience simultaneous onset of ocular and cutaneous manifestations. Importantly, the severity of ocular rosacea does not correlate with the severity of skin disease.